



Employee Welfare Benefits

Application Form
(Applicable for SMT Members)

Employee Name	Mr. Rajaram . m	Employee Code	701700		
Date of Joining Apollo (DD-MM-YYYY)	09052011	Service in Apollo (Yrs)	15 years		
Mobile Phone No.	+ 9 1 9 6 2 9 6 4 5 1 8 3	Department & Designation	BO-8 / SMT		
Type of Welfare Benefits Sought (Tick in the relevant box)	Marriage ☐	Child Birth Child 1 ☒ Child 2 ☒	Paternity Leave ☒	Child School Admission ☐	Housing Assistance ☐
Dates (Fill only relevant box)	Date of Marriage 0 9 0 5 2 0 1 1	Date of Delivery 1 1 0 8 2 0 2 6	From 1 0 0 8 To 1 8 0 8	Date of Admission 0 9 0 5 2 0 1 1	Date of Regn. 0 9 0 5 2 0 1 1
Gift Amount (INR)	5,000 / 50,000	25,000	NA	25,000	1,00,000
Proof Attached (Fill only relevant box)	Wedding Card or Regn. Certificate	Discharge Summary or Birth Certificate	☒ Discharge Summary	School Fee Receipt	Registration Proof
Additional information required for availing insurance benefits	Spouse Name Spouse DOB 09041996	Child Name Vela Varshini Child DOB 11082026	Apply for Paternity leave Applied for Sparsh		

1. I hereby certify that the above Information is true and declare that I have not availed any of the benefits* earlier.
2. I added my Spouse/Child details in SPARSH and agree to pay the applicable additional premium for Mediclaim Insurance Policy..

Employee's Signature : **Rajaram**

Date : **17/08/2026**

FOR HR USE ONLY

Approved by
Divisional Head - HR :

FOR ACCOUNTS USE ONLY

Signature & Date of Receipt of Form

Note :

Applicable to All Employees

* Nine calendar days can be taken, consecutively, after the childbirth as per the requirement of the employee (3 Days before or after the delivery day shall be permitted).

Applicable only to SMT Members

- Employees with service of 4 Yrs & above as on Marriage Day, can avail Marriage Gift by applying on SPARSH - An Employee Portal
- Employees with service of 5 years and above as on Child Delivery Day, can avail Child Birth Gift.
- Employees with service of 7 years and above as on School Admission Day, can avail Child School Admission Gift.
- Employees with service of 10 years and above can avail Housing Assistance. However if house is bought / built after 4 years of service, benefit can be availed only after completion of 10 years of service. Prerequisite is property should be in the name of either employee or jointly with employee's spouse or jointly with employee's parent and should be within the radius of 80 Km from the Plant.

Filled in application form to be submitted in HR-Help Desk, 15 days in advance from the date of marriage/child birth/school admission/house regn.

Form No. CH / HR / 0004A

Received on **17/08/26**



Hindu Mission Hospital

NABH / NABL Accredited

103, GST. Road, Tambaram (West), Chennai-45.

ISO 9001 : 2008 Certified Hospital



HMH/MR-97/2024/10000

Ph No. : 2226 2244 Fax No.: 2226 2255

Website : www.hindumissionhospital.org

Email : hmh@dataone.in

DISCHARGE SUMMARY

GENERAL INFORMATION:

NAME: Mrs. JAYALAKSHMI.J

AGE: 30 Y 4 M 4 D SEX: Female

UHID NO: HMH7116369

IP NO: IPV2026006636

PRIMARY CONSULTANT: DR. POOVIZHI

(OB / GYNAECOLOGY)

CO-CONSULTANT: DR. VAISHNAVI (INTERNAL MEDICINE)

D.O.Admission: 10-08-2026

D.O.Discharge: 14-08-2026

Reason for Admission:

OBS SCORE OF G2P1L1 ADMITTED FOR SAFE CONFINEMENT

DIAGNOSIS:

G2P1L1>>>P2L2

NVD WITH EPISIOTOMY- VIA KIWI ASSISTED VAGINAL DELIVERY ON 11-08-2026

CHIEF COMPLAINTS:

G2P1L1 AT 39 WEEKS + 4 DAYS OF GA ADMITTED FOR SAFE CONFINEMENT

HISTORY OF PRESENT ILLNESS:

A 30 YEAR OLD FEMALE G2P1L1 WITH 39 WEEKS + 4 DAYS OF AMENORRHOEA PREV NVD ADMITTED FOR SAFE CONFINEMENT

NO H/O ABDOMINAL PAIN

NO H/O LEAKING PV

NO H/O BLEEDING PV

NO H/O FEVER, COLD AND COUGH

PATIENT ABLE TO PERCEIVE FETAL MOVEMENTS

H/O INJ IRON SUCROSE 2 DOSES GIVEN

PAST HISTORY:

NIL

H/O TONSILLECTOMY DONE 1 YEAR BACK REPORTS NA

H/O NASAL POLYPECTOMY DONE 1 YEAR BACK REPORTS NA

DRUG ALLERGY:

NO KNOWN ALLERGY

TAL

10.15
57'

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CLINICAL EXAMINATION FINDING:

OE:

CONSCIOUS

ORIENTED

AFEBRILE

NO PALLOR/ PE

SE:

CVS: S1S2+

RS:BAE

PA: SOFT, ACC TO GA

CNS: NFND

VITALS

BP: 120/80 MMHG

PR: 98/MIN

SPO2: 98% AT RA

TEMP: NORMAL

FHR: 153/ MIN

INVESTIGATIONS:

10-08-2026 : Creatinine : 0.6 mg/dl; Bilirubin Total : 0.5 mg/dl; Bilirubin Direct : 0.2 mg/dl; Bilirubin Indirect : 0.3 mg/dl; Alk Phosphatase : 128 U/L; Total Protein : 5.7 gm/dL; Albumin : 3.2 gm/dL; Globulin : 2.5 gm/dL; AG Ratio : 1.28 gm/dL; Gamma Glutamyl transferase : 10.2 U/L; SGOT/AST : 16 U/L; SGPT/ALT : 12 U/L; UREA : 19 mg/dl;

11-08-2026 : RBC : 3.25 mill/cu.mm; HAEMOGLOBIN : 10.1 gm/dL; PCV : 30.4 %; MCV : 94 fL; MCH : 31 pg; MCHC : 33.1 g/dL; TOTAL WBC Count : 19 Cell/mm³; NEUTROPHIL : 84.7 %; LYMPHOCYTE : 11.4 %; EOSINOPHIL : 1.5 %; MONOCYTE : 2.1 %; BASOPHIL : 0.3 %; PLATELET COUNT : 1.38 lakhs/cu.mm; ESR 1 HOUR : 32 mm/hr; RDW : 12.2 %; RBC : Sparsely distributed RBCs. Normocytic normochromic RBCs seen. ; WBC : Total count elevated, differential count shows neutrophilic predominance with shift to left upto metamyelocytes. Few hypersegmented neutrophils seen. ; Platelet : Adequate on smear. Few small clumps seen. ; Immature : Seen upto metamyelocytes. ; MP and MF : Negative ; IMPRESSION : Moderate normocytic normochromic anemia with neutrophilic leucocytosis with shift to left upto metamyelocytes. ;

12-08-2026 : Physical Examination : ; Color : Pale Yellow ; Appearance : Clear ; Chemical Examination : ; Reaction : Acidic ; pH : 6.0 ; Specific Gravity : 1.005 ; Protein : Negative ; Sugar : Negative ; Ketones : Negative ; Bilirubin : Negative ; Urobilinogen : Normal ; Blood : Negative ; Leukocyte Esterase : Negative ; Nitrite : Negative ; Microscopic Examination(Light Microscope) : ; WBC : 1-2 ; Epithelial Cells : 1-2 ; RBC : Nil ; Casts : Nil ; Crystals : No crystals seen ; Yeast Cells : Not Present ; Bacteria : Not Present ; Crystals_1 : ; Others : Not Present ;



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REVIEW:

REVIEW WITH DR. POOVIZHI (GYNAECOLOGY) AFTER 1 WEEK IN OPD/SOS

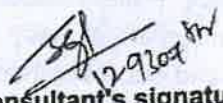
REVIEW WITH DR.VAISHNAVI (GENERAL PHYSICIAN) WITH URINE C/S, VAGINAL C/S AFTER 1 WEEK IN OPD AFTER 1 WEEK

WHEN TO OBTAIN EMERGENCY CARE AFTER DISCHARGE:

- **Fever of 101 degree F**
- **Onset of new pain or worsening of previous pain**
- **Vomiting**
- **Breathing difficulty**
- **Altered level of conscious**

HOW TO OBTAIN EMERGENCY CARE:

CALL: 8754595006, 044-22262244 AND ASK THE OPERATOR TO CONNECT THE DOCTOR & CALL FOR AMBULANCE.


Consultant's signature:



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13-08-2026 : RBC : 3.85 mill/cu.mm; HAEMOGLOBIN : 11.8 gm/dL; PCV : 35.8 %; MCV : 93 fL; MCH : 30.8 pg; MCHC : 33.1 g/dL; TOTAL WBC Count : 12 Cell/mm³; NEUTROPHIL : 71.3 %; LYMPHOCYTE : 24.4 %; EOSINOPHIL : 2.2 %; MONOCYTE : 1.5 %; BASOPHIL : 0.6 %; PLATELET COUNT : 2.06 lakhs/cu.mm; ESR 1 HOUR : 36 mm/hr; RDW : 12.3 %; RBC : Normocytic normochromic RBCs seen. ; WBC : Total count increased and differential count shows neutrophilic predominance ; Platelet : Adequate ; Immature : Nil ; MP and MF : Negative ; IMPRESSION : Mild Normocytic normochromic anemia with Neutrophilic leucocytosis. ; PROCALCITONIN : 0.55 ng/ml;

COURSE IN THE HOSPITAL & DISCUSSION:

Patient was admitted with the above complaints. Relevant investigations were done CBC done on 11/08/26 showed- Moderate normocytic normochromic anemia with neutrophilic leucocytosis with shift to left upto metamyelocytes. Obstetrician opinion were obtained and advised admission for safe confinement. Patient undergone kiwi assisted vaginal delivery with episiotomy . patient delivered an alive term female of baby weight 3 kg at 12.31 pm placenta and membrane removed in toto . cord clamped and cut, baby handed over to pediatrician .episiotomy wound closed in layers lower segment of uterus failed to contract packing done , with utero tonics , bladder catheterized ,uterus firm and contracting well. 12.08.2026 Obstetrician reviewed and advised for general physician opinion, high vaginal smear taken and added medications . General Physician opinion obtained and advised to do rpt cbc with mannual platelet count, inspite of increased Total count plan for serum procalcitonin to be done and to do midstream urine routine and culture and sensitivity , high vaginal swab culture and sensitivity and if platelet drops plan dengue ns1, scrub typhus . General Physician reviewed the patient and advised to continue the same medications . On 14/08/2026 patient was reviewed by gynecologist, patient was stable hence advised discharge.

Patient was treated with IV Antibiotics, PPI, Anti emetics, Analgesics, and other supportive medications. Patient improved symptomatically better hence discharged. On discharge patient vitals are normal.

DISCHARGE ADVICE:&DRUGS:

SN O	Drug	Dose	Dosage Form	Route	Frequency	Timing	Duration	Comments
1.	TAXIM O	200MG	TAB	PO	1-0-1	AF	5 DAYS	
2	PAN	40MG	TAB	PO	1-0-0	BF	5 DAYS	
3.	ACTON OR	1 GM	TAB	PO	1-0-1	AF	5 DAYS	
4.	FERRONOMIC PLUS	1 TAB	TAB	PO	1-0-0	AF	3 MONTHS	
5.	SHELCAL	500MG	TAB	PO	0-1-0	AF	3 MONTHS	